

| 1986 | FIRST IVF BABY

## Test Tube Triumph

Harsha's miracle continues to offer hope to disappointed parents

When I started, I did not even know what an egg looked like," recalls Indira Hinduja, the gynaecologist responsible for the birth of Harsha Chawda, India's first IVF baby in 1986. "She was something impossible when she came through but it was a thrilling experience which made me feel that I had finally done something worthwhile indigenously," says Hinduja. The birth of the girl at the state-run hospital K.E.M. Hospital in Mumbai became instant news. Harsha's birth opened up the much-sought opportunity for treating couples incapable of natural reproduction. The Indian Council of Medical Research (ICMR) estimates at least 10 per cent of couples in India face infertility. Apart from factors like low sperm count, infections and erectile dysfunction in males, damaged fallopian tubes, low egg production and fibroids in females, lifestyle changes have also adversely affected fertility. In Vitro Fertilisation (IVF) is a complex process in which the ovum is fertilised outside the body and the fertilised egg is then implanted to the uterus. Research in Assisted Reproductive Technology, which includes Artificial Insemination, In Vitro Fertilisation-Embryo Transfer and Intracytoplasmic Sperm Injection, has grown exponentially. Today, eggs are frozen for future use, embryos screened for genetic disorders and databases developed to help find ways to minimise miscarriages. Presently, there are more than 400 IVF clinics in India that even treat those coming from abroad due to the comparatively lower costs and, in some cases, for Indian donor eggs. Hinduja has delivered about 5,000 babies through assisted reproduction since Harsha, who is now an office assistant with Central Investigation Security Service, a private security agency in Mumbai. Harsha arrived eight years after the birth of 'Durga', later known as Kanupriya Agarwal, in Kolkata whose birth inspired by IVF techniques was mired in ethical and moral controversies. The West Bengal government denounced Dr Subhash Mukhopadhyay's claim that he had created history. In frustration, he committed suicide in 1981. Finally, in 2005, the ICMR recognised his contribution and upheld his claim. It was tragically late. **by Amarnath K. Menon**

HARSHA CHAWDA



SAARC LEADERS AT THE NANDI HILLS

### THE MAIN EVENT

#### SAARC Summit-II

The road to SAARC summits are paved with good intentions, but it is the permanent potholes that succeed in ensuring that any progress gets bone-jarring and painfully slow. Collectively, the seven-nation body (India, Bangladesh, Bhutan, Sri Lanka, Pakistan, Nepal and the Maldives) constitutes the single most populous (997.85 million) and poorest (average per capital income: US \$241) grouping in the international community. With two dictatorships, two monarchies and three democracies, it is also the most politically divergent. The two most powerful members, India and Pakistan, with their ingrained attitude of mutual mistrust and hostility, cannot resist the temptation to use SAARC as a stage for political one-upmanship which does much to dilute its credibility.

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#### ALSO

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**THE NUMBER OF IVF TREATMENT CYCLES HAS GONE UP FROM 7,273 IN 2001 TO 19,005 IN 2005.**